

THE FRONTLINE CASE — IN THE SPACE BETWEEN POLICY SERIES

PROTECTING THE PROTECTORS.

The frontline case for a Healthcare Special Constable designation in Ontario: identity, evidence, retention, fiscal context, proof of concept, and the specific implementation pathway — written from the floor and handed off to the policy specification that follows.

THESIS

The people already doing this work are the least legislatively supported professional workforce on Canadian public safety's frontline. The fix exists. It is operational in Alberta, available under Ontario's Community Safety and Policing Act, and already running in Ottawa for OC Transpo. Applying it to Ontario hospitals is what this paper argues for.

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PAPER

II of III

FORMAT

Frontline Case

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SUBMITTED TO

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Where This Paper Sits.

// Paper I

Hold the Line

A PhD-level editorial on the emerging mass casualty risk in Canadian hospitals and the professional workforce positioned to prevent it. Written to win attention and build moral consensus.

EDITORIAL · PUBLIC-
FACING

// Paper II — You Are
Here

Protecting the Protectors

The frontline case for the Healthcare Special Constable designation. Identity, evidence, retention, fiscal context, Alberta precedent, training, and a bipartisan implementation framework.

FRONTLINE CASE · THE
BRIDGE

// Paper III

The Space Between

The full fifty-four-page policy specification. Three-stage failure model, Charter analysis, clinical primacy enforcement framework, Ottawa pilot design, and binding evaluation criteria.

POLICY REPORT · THE
SPECIFICATION

The System Is Strained. The Fix Already Exists in Statute.

Ontario's hospital Protection Services workforce operates, every shift, at the outer edge of what its current legal authority allows. The role manages mental health crises, weapons encounters, Code White events, and prolonged police-handover situations using authority primarily derived from the *Criminal Code's* citizen's arrest provision and the *Trespass to Property Act*. That authority was designed for shopping centres and office buildings. It was not designed for the operating environment of a twenty-first-century Canadian emergency department.

This paper is the frontline case for closing that gap. It sits between the *Hold the Line* editorial (Paper I) — which documents the incident record — and *The Space Between* policy report (Paper III), which specifies the legislative, Charter, clinical, and fiscal framework in full. This paper is the bridge between the case for urgency and the case for the specific mechanism.

+47%

Increase in Ontario ED visits for mental health, addictions, and substance use; youth 14–17 hospitalisations rose 136%.

// CMHA State of Mental Health in Canada, 2024

+15%

Growth in Ottawa Police Service mental health calls since 2019. Occurrences with a mental health component rose 30%.

// OPS Crisis Intervention Team Program Report, 2025

70%

Of 460+ Canadians who died in police encounters from 2000 to 2017 had mental health or substance use issues.

// CBC News Deadly Force database, 2018

170

WorkSafeBC accepted injury claims for BC hospital security guards in a single year — approximately one every other day.

// CBC News / WorkSafeBC, August 2025

Alberta has addressed an equivalent gap through a Community Peace Officer designation operational for over a decade within its integrated provincial health system. British Columbia is actively discussing it — the Victoria Police Chief has named it publicly, the Union of BC Municipalities has passed a resolution calling for the enabling legislation, and the Paul Spencer Coroner's inquest has examined the adverse outcomes that occur in its absence. Ontario has the mechanism — CSPA Part XIV Special Constable designations — already operating in Ottawa through the OC Transpo Special Constable Unit. What has not yet been done is the application of that mechanism to the hospital environment.

This paper argues for that application, on frontline evidence, at a fiscal envelope consistent with Ontario's current constraints, with a pilot designed to prove or disprove the model through binding evaluation criteria. The detailed specification is in Paper III.

We Are Not Security Guards.

And the reason we are still called that is part of the problem.

The title problem is not a question of professional pride. It is a regulatory and legislative problem with operational consequences. The word "security guard" has a specific meaning in Ontario law — a meaning that does not match the work being done by in-house hospital Protection Services in any major Ontario acute care setting today.

Ontario's *Private Security and Investigative Services Act* regulates the "security guard" category as a function of asset protection, access control, and observation. That framework was built for commercial premises, event security, and property patrol. It was not built for a role that manages agitated psychiatric patients in a clinical setting, assists clinical staff in physical interventions, conducts perimeter control during active lockdowns, and stands as the first uniformed presence at every Section 17 police handover.

What We Actually Do

On any given shift at a major Ontario hospital, a Protection Services Officer may respond to a patient in acute psychiatric crisis; assist clinical staff during a Code White; manage a domestic conflict that has followed a patient into the waiting room; secure the perimeter during a lockdown; provide first-response assistance during a cardiac event until clinical staff arrive; and stand in a parking lot next to a vehicle with a known weapon inside, in the dark, waiting for police who may or may not arrive within a clinically relevant window.

We do this alongside physicians, nurses, paramedics, and social workers as a functional component of the hospital's care and safety infrastructure. We are trained in de-escalation, crisis intervention, restraint and control, emergency response, and use of force. We hold institutional authority as agents of the property occupier. We are, routinely, the first trained responder on scene when something goes wrong.

The law has not kept up with this reality. This paper explains why that needs to change — and exactly how.

The Legislative Gap

Under **Section 494 of the Criminal Code of Canada**, Protection Services Officers hold citizen's arrest authority. As occupiers of hospital property under the *Trespass to Property Act*, this extends to situations where a criminal offence is being committed or is clearly and imminently about to occur on the premises. This is the full scope of the legislated authority we carry, and we exercise it correctly every shift.

The critical limitation: **mental health crisis does not begin as a crime.**

The authority to apprehend a person for psychiatric assessment under **Section 17 of the Ontario Mental Health Act** belongs exclusively to police officers. Not to nurses. Not to paramedics unless under specific delegation. And not to Protection Services Officers, regardless of training, proximity to the situation, or the clinical resources standing immediately available to receive the patient.

The result is a response window — often measured in hours — during which a person in acute mental health crisis occupies hospital property, presents documented risk to themselves and others, and remains completely outside the lawful reach of the trained professionals directly in front of them.

// COMPOSITE SCENARIO — DRAWN FROM DOCUMENTED PATTERN

A person arrives in acute psychiatric and behavioural crisis. Suicidal ideation is expressed openly. Dangerous behaviour is exhibited before law enforcement arrives. A physical altercation occurs requiring security intervention before police are contacted. The person is placed in a vehicle in the hospital parking lot to wait. During that wait, it is disclosed that a bladed weapon is inside the vehicle. The person is alone in the vehicle, agitated, in an altered state, and police response is approximately ninety minutes out. Protection Services Officers observe from a distance, with no legal authority to intervene on mental health grounds, no defensive tools appropriate to a potential weapons encounter, and no escalation pathway that does not involve waiting for police. The situation ultimately resolves not through mental health intervention but through arrest, triggered hours later when the person's crisis has deteriorated further. They are removed in handcuffs rather than care. The healthcare system has failed the person. The protection services team has been placed at an unacceptable level of risk. And the only reason the worst did not happen is circumstance.

We can act after a crime. We cannot act to prevent the crisis that leads to one. That gap is not theoretical. It is documented, recurring, and closing faster than the legislation governing it.

// TAKEAWAY

The title problem is a regulatory mismatch with operational consequences. The fix is to match the regulatory framework to the work that is actually being done.

// 02 — THE EVIDENCE BASE

The Sources Line Up.

Ottawa — The Local Picture

Ottawa Police Service published data directly supports the systemic nature of this problem at the local level. Mental health calls for service in Ottawa have increased approximately 15% since 2019. Occurrences with a mental health component have risen approximately 30% over the same period. In response, OPS launched its Crisis Intervention Team (CIT) program in 2025 — an acknowledgment in itself that existing response models have become insufficient.

The CIT program also provides a concrete measure of what special constable designations produce when applied correctly. The OPS special constable expansion, funded by a **\$4 million grant from the Ontario Ministry of the Solicitor General**, freed an average of **250 officer-hours per platoon** by replacing sworn officers at lower-priority calls, including hospital guarding duties. Each relieved officer attended an average of **2.5 additional calls per day**. This is the efficiency case for the Special Constable model, in concrete published terms. It is not modelled. It is measured. And it applies directly to the hospital handover context — a category the OPS CIT pilot has already partially captured, and which a dedicated Healthcare Special Constable designation would capture in full.

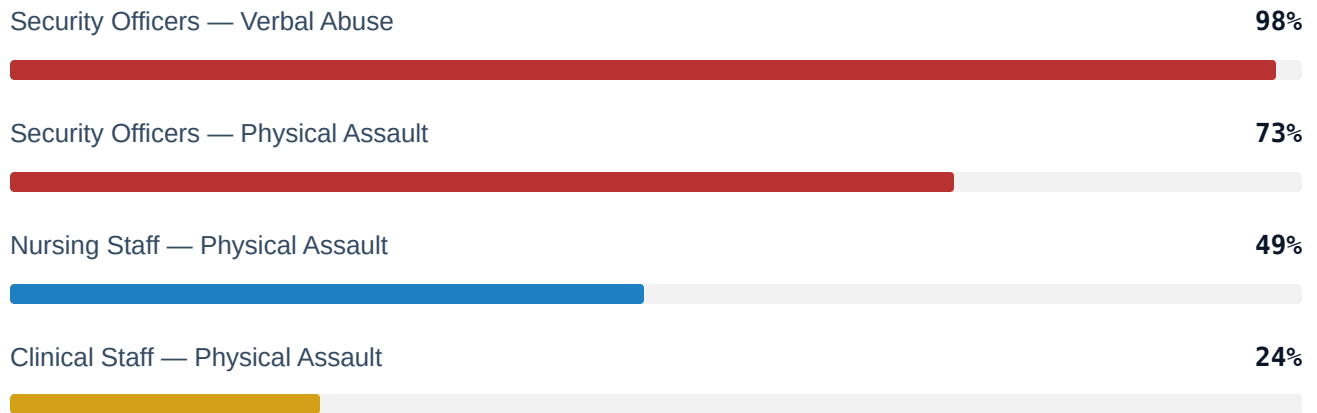
The Provincial Escalation

Ottawa is not an anomaly. It is a data point in a provincial trajectory that has been building for years and accelerated significantly following the pandemic.

- **Emergency department visits** for mental health, addictions, and substance use in Ontario have increased approximately **47%**, with a **136% increase in psychiatric hospitalisations among youth aged 14–17** (CMHA, *State of Mental Health in Canada*, 2024).
- **Kurdyak et al. (JAMA Network Open, 2021)** documented in a cohort study of **659,084 Ontario psychiatric ED visits** that **45.4% were first-contact** — no prior outpatient mental health care in the preceding two years.
- **Fung and Kim (European Psychiatry, 2024)** documented in a study of **9,848 Ontario psychiatric inpatients** that **involuntary admissions reached 55.7%** in 2021, up 6.8 percentage points year over year.
- **CIHI NACRS** documented more than **16.1 million unscheduled emergency department visits in Canada in 2024–25**, up from approximately 15.5 million the previous year.

Violence Exposure at the Role Level

A peer-reviewed study published in 2021 examining violence exposure across occupational groups in a Canadian emergency department setting found that security officers recorded both the highest rate of verbal abuse and the highest rate of physical assault of any category in the ED workforce.

Security officers absorb the highest assault rates of any ED worker category studied.

// Source: "Unheard Victims: Multidisciplinary Incidence and Reporting of Violence in an Emergency Department" — peer-reviewed, PMC, 2021

This pattern is reinforced in national workforce reporting. The Canadian Federation of Nurses Unions, in its 2025 report *Violence against Nurses in Canada: An Urgent Call to Action*, found that approximately **six in ten nurses** reported experiencing violence at work over a one-year period. The CFNU's first campaign to eliminate healthcare workplace violence was in 1991. The trajectory, over three decades, has been upward.

In British Columbia, where equivalent data has been systematically captured, **WorkSafeBC accepted approximately 170 hospital security injury claims in a single recent year** — described in reporting as "injured nearly every other day" for the province's hospital security workforce. BC Nurses' Union president Adriane Gear has described an average of **46 monthly WorkSafeBC claims** from nurses for workplace violence requiring time off. Ontario does not systematically publish comparable data. That absence of data is itself a policy problem, and one this paper recommends correcting as part of the pilot's data collection framework.

The Police Handover Problem

A documented pattern in Ontario's mental health system is what researchers have characterised as "police handover drift" — the practice of police bringing individuals in mental health crisis to emergency departments and, under pressure of competing operational priorities, departing before clinical handoff is fully completed. This is not a criticism of individual officers. It is the systemic consequence of assigning a sustained hospital-based clinical-bridging function to a service whose mandate and capacity are designed for community response.

The **Provincial HSJCC Police-Hospital Transition Framework** — endorsed by the Ontario Ministry of the Solicitor General and the Ministry of Health in 2019 — acknowledged these failures directly. That framework, however, governs transition processes. It does not address the fundamental authority gap that creates the problem in the first place: the absence, on hospital property, of a continuously present professional who holds the legislative authority to bridge a person in crisis to clinical assessment.

The CBC News *Deadly Force* investigation — the first comprehensive Canadian database of police-involved fatalities — documented **more than 460 such deaths in Canada between 2000 and 2017**, and found that **70% of those who died had mental health or substance use issues or both**. The Tracking (In)Justice project, building on the CBC dataset, has documented that the number of fatal encounters with police has continued to rise through 2022. The Canadian Association of Chiefs of Police has itself acknowledged that police should not be, by default, the frontline for mental health crises. An alternative in the hospital context is a trained, legislatively empowered Protection Services professional whose authority allows them to act on-site before the crisis requires full police involvement at all.

// TAKEAWAY

The evidence base is not thin. It is publicly available, peer-reviewed in places, and institutionally sourced. It points, across every category examined, to the same structural conclusion.

// 03 — THE RETENTION PROBLEM

Why the Best People Leave.

The standard explanation for turnover in hospital Protection Services — "it doesn't pay enough" — is incomplete and, at many in-house hospital sites, factually incorrect.

The OC Transpo Special Constable role in Ottawa, widely used as a compensation benchmark for comparable positions in the city, pays approximately **\$80,893 to \$95,168 annually** as of 2025. In-house hospital Protection Services rates at major Ottawa acute care facilities are in a comparable range. Compensation is not the primary driver of departure at established in-house programs. What follows is what actually drives the best people out of the profession.

They Leave for Police

Hospital Protection Services functions as a de facto pipeline to municipal policing. The operational experience, crisis management skills, de-escalation training, and exposure to high-stakes clinical scenarios that this work provides are exactly what police services are looking for in recruits. Security contractors explicitly market hospital experience as police career preparation — and they are correct to do so. Hospital Protection Services personnel are exceptionally well-prepared for policing careers.

The problem is that the profession loses its most capable, most experienced, and most invested people to a career pathway that recognises their skills as worthy of real authority, real equipment, and real institutional backing. The message the current framework sends is that those skills are worth more in a different uniform than the one being worn when they were developed.

They Leave Because the System Signals They Are Replaceable

When an employer calls a decade-trained Protection Services Officer a "security guard" in formal documentation. When institutional investment in professional development stops at the minimum required for the contract. When the response team managing the highest-risk calls on shift is the last to receive upgraded equipment. When a decade of developed expertise is treated as interchangeable with a hire from last week — people with options exercise them. The ones without options stay and, over time, burn out.

They Leave Because of Moral Injury

This factor is underreported and critically important. Protection Services professionals in hospital settings witness more unresolved human suffering than almost any other non-clinical role in the building. When the system consistently fails to provide appropriate care to people in crisis — when the same individuals cycle back in progressively worse condition, when an officer stands helpless in the gap between a crisis and a crime because the law will not permit bridging it — that takes a measurable toll.

CIHI data documents more than **120,000 healthcare vacancies in Canada in 2022–23**, with burnout identified as a primary driver of early departure across healthcare professions. The **Ontario Council of Hospital Unions** documented a **331% increase in hospital job vacancies since 2015**. Hospital Protection Services is not immune to this dynamic. It is, arguably, the frontline most exposed to it.

THE SOLUTION IS NOT A WAGE INCREASE. IT IS PROFESSIONAL LEGITIMACY.

A Healthcare Special Constable designation does something no compensation adjustment can: it signals, through legislation, that the province recognises this role as a professional designation requiring real authority, real training standards, real equipment, and real institutional support. That signal matters more to retention than dollars, because it addresses the actual driver of departure.

If Ontario wants to retain the experienced, high-calibre Protection Services professionals its hospitals already depend on, the most effective intervention is not compensation. It is a legislated professional role that is *designed* for the healthcare environment — rather than a regulatory framework designed for a different setting that the hospital context is asked to tolerate.

Ontario Cannot Afford Inaction.

Any policy proposal that ignores Ontario's current fiscal reality is not worth reading. This one does not.

The FAO's Documented Funding Gap

The province's **Financial Accountability Office**, in its *Ontario Health Sector: 2025 Spending Plan Review* (published October 2025), documented the following:

- The 2025 Ontario Budget projects health sector spending to grow at an **average annual rate of 0.7%** from 2024–25 to 2027–28, from \$91.6 billion to \$93.6 billion.
- The FAO estimates that **4.0% annual growth is required simply to maintain current service levels**, based on its cost-driver analysis (population growth, population aging, and healthcare inflation).
- The resulting funding shortfall is **\$3.4 billion in 2025–26, \$6.4 billion in 2026–27, and \$9.6 billion in 2027–28.**

- If achieved, the 2025 budget's health sector spending plan would be the slowest three-year growth rate since 1993–94 to 1996–97.

Against that backdrop, every dollar committed to new healthcare infrastructure must demonstrate either direct service improvement or offsetting efficiency. The Healthcare Special Constable proposal is designed to meet both tests.

The Three-Part Fiscal Case

EARLIER INTERVENTION REDUCES TOTAL SYSTEM COST

A person apprehended promptly by a trained, legislatively empowered hospital officer and assessed quickly by a physician costs the system less than the same person cycling through three ED visits, a police response, and a period of pre-charge custody before eventually accessing care. The cost avoidance math is directionally clear even without precise Ontario-specific modelling — and the specific modelling is developed in detail in Paper III.

REALLOCATION OF EXISTING CAPACITY, NOT NET NEW PUBLIC SPENDING

Protection Services Officers are already deployed in hospitals. They are already managing these situations to the limit of their legal authority. The incremental cost of a Special Constable legislative designation, combined with an enhanced training program, is a fraction of the cost of building parallel response infrastructure from scratch. The Alberta model operates within existing hospital budget structures, not as a standalone line item. The proposed Ontario pilot is designed to do the same.

POLICE RESOURCE RECOVERY

The OPS CIT data is the cleanest available evidence: 250 officer-hours freed per platoon, 2.5 additional calls per day per officer relieved, funded by a \$4M Solicitor General grant that is itself formal provincial acknowledgment of the scale of the reallocation opportunity. Applied to hospital settings, the same mechanism recovers police capacity for calls that genuinely require full police authority. This is a net efficiency for the municipal public safety system, not a budget transfer.

Why Ottawa — Not Toronto — Should Lead This Pilot

When Ontario launches a healthcare policy pilot, the default assumption is Toronto. Toronto has the volume, the institutional profile, and the political gravity. Most major provincial healthcare initiatives of the past decade have started in the GTA and worked outward.

This one should be different — and here is the case for why Ottawa is the superior choice for a two-site pilot.

- **Ottawa is where the legislative precedent already exists.** The OC Transpo Special Constable Unit — the operational template for the Hospital Special Constable designation — is an Ottawa institution, appointed by the Ottawa Police Services Board under the CSPA. The legal, administrative, and training infrastructure does not need to be built in Ottawa. It needs to be extended.
- **Ottawa is where the mental health pressure on police capacity is documented at public-record level.** The 30% increase in occurrences with a mental health component since 2019, paired with the \$4M Solicitor General grant and the CIT launch, creates the clearest documented Ontario evidence environment for a hospital pilot.
- **Ottawa is the national capital.** A pilot that succeeds here succeeds in front of federal MPs, provincial MPPs, municipal councillors, national media, and the senior civil service. Visibility of success in Ottawa is materially greater than visibility of success in a comparably-sized Ontario city.

SITE ONE — QUEENSWAY CARLETON HOSPITAL

QCH is the sole full-service acute care hospital serving a population of over 500,000 in west Ottawa. It holds Accreditation Canada's Accreditation with Exemplary Standing, with a 99.4% compliance score across the Required Organizational Practices. Through the \$6 million Hopes Rising Campaign, QCH has opened the Barbara Crook and Dan Greenberg Mental Health Centre, expanding its mental health services footprint. The pilot case for QCH is a community acute care model: sole acute facility for a large west-Ottawa catchment, in-house Protection Services team suited as Tier 1 grandfather candidates under the proposed designation framework, minimal capital requirement, and a controllable evidence dataset.

SITE TWO — THE OTTAWA HOSPITAL

TOH operates two full emergency departments at its Civic and General campuses, each seeing approximately **210–230 patients per day**. TOH maintains two of Ottawa's four adult Psychiatric Emergency Services, holds 96 inpatient mental health beds, and is the city's largest provider of acute mental health care. Its own planning documents project a significant increase in yearly ED visits over the coming years. A Healthcare Special Constable pilot at TOH would generate the highest-volume, most complex dataset available in Eastern Ontario — in a bilingual, research-affiliated environment.

The combination of QCH (community acute care model) and TOH (major teaching hospital model) gives the province two distinct implementation environments in the same city. The comparative dataset is stronger than any single Toronto-based pilot could produce. The nation's capital has the institutions, the

infrastructure, the operational precedent, and the professionals to lead.

// TAKEAWAY

The fiscal case does not ask the province to spend more in a constrained health budget. It asks the province to spend what is already being spent more efficiently, and to allow a documented offset mechanism to recover capacity at the margin.

// 05 – THE PROOF OF CONCEPT

Alberta Has Been Doing This for Over a Decade.

The Alberta Health Services Protective Services / Community Peace Officer program is not a concept paper. It is a fully operational, multi-site program that has been running across one of Canada's largest integrated provincial health systems — a system serving nearly five million Albertans — for more than ten years. For readers unfamiliar with the AHS model, here is what it is and how it differs from the current Ontario approach.

What an AHS Peace Officer Is

An AHS Protective Services Officer is appointed under the **Alberta Peace Officer Act, RSA 2006, c P-3.5**, and designated as a Community Peace Officer by the Alberta Solicitor General, with specific enforcement authorities defined by that appointment. These officers carry legislative authority to act on mental health presentations in the hospital environment before clinical assessment has been initiated — operating under a defined provincial framework that specifies training standards, use of force, and oversight.

Critically: AHS peace officers are not police. They do not investigate crimes, carry firearms, patrol communities, or hold broad law enforcement authority. Their jurisdiction is defined and bounded by AHS property and mandate. They are healthcare-environment professionals with a legislative framework that reflects the actual demands of the environment.

The training pathway is a **six-week paid induction program**, jointly administered by Alberta Health Services and the Alberta Justice and Solicitor General. Training covers applicable legislation, crisis intervention, control tactics, investigative procedures, emergency response, and communication. Officers then complete a supervised field training program before full deployment.

The Liability Clarification

One of the most significant and underappreciated dimensions of the AHS model is what it does to the institutional liability structure. Under the current Ontario framework, when a Protection Services Officer acts — or fails to act — in a mental health crisis, the legal exposure flows back to the hospital as employer under the occupier model and applicable employment law.

Under a Special Constable designation, the officer's authority derives from a provincial legislative appointment with defined oversight (CSPA Part XIV oversight, Law Enforcement Complaints Agency jurisdiction, Ottawa Police Services Board review, and, under the pilot proposed in Paper III, independent academic evaluation). The framework shifts from a purely employer-employee model to one that includes defined provincial regulatory oversight, analogous to the OC Transpo Special Constable structure.

This does not eliminate employer responsibility. It creates a defined legislated framework for the exercise of authority that significantly clarifies the liability landscape for hospital administrators. In plain terms: a hospital CEO whose Protection Services team operates under a provincially-legislated Special Constable designation is in a substantially different legal position than a hospital CEO whose team is managing mental health crises under a citizen's arrest framework that was designed for a different setting entirely.

CURRENT ONTARIO MODEL

Protection Services Officer

- Section 494 *Criminal Code* authority — criminal conduct only
- Occupier authority under the *Trespass to Property Act*
- No authority under the Ontario *Mental Health Act*
- Dependent on police for MHA apprehension authority
- No standardised defensive equipment framework
- Liability structure flows to hospital as employer

PROPOSED ONTARIO MODEL

Healthcare Special Constable

- All existing criminal and occupier authorities retained
- CSPA Part XIV Special Constable designation
- Authority to initiate Section 17 apprehension on hospital property
- Defined provincial training and equipment standards
- Clinical primacy framework and defined oversight structure
- Shared liability model with provincial regulatory framework

Alberta vs Ontario — The Healthcare Context

Alberta operates as a single provincial health authority. Ontario operates through a network of independent hospital corporations with Ontario Health providing system-level coordination. This difference affects implementation design but not the fundamental argument.

Both provinces face rising mental health call volumes. Both face police capacity constraints. Both have hospital Protection Services professionals managing situations that exceed their current legislative authority on a daily basis. Alberta identified this and created a legislated solution. Ontario has not — yet. The distinction is not operational complexity. It is legislative will.

If anything, Ontario's hospital-specific model makes the Special Constable designation *simpler* to implement, because the existing provincial Special Constable framework under the **Community Safety and Policing Act, 2019** already provides the legal mechanism. OC Transpo's Special Constables operate under exactly this framework, appointed by the Ottawa Police Services Board with Solicitor General approval. Hospital Special Constables can operate under an equivalent model, with hospitals as the employing entity in the same way OC Transpo functions as the employing entity for its transit constables.

Training That Matches the Work.

"Better training" is the generic policy response to every frontline safety problem, and it is frequently unspecified. What follows is a specific, costed, precedent-backed training framework designed for Ontario's fiscal environment and built on models already operational in this city.

The OC Transpo Special Constable Benchmark

OC Transpo Special Constables in Ottawa complete an 8-week post-hire induction program plus supervised home-service training, appointed under CSPA Part XIV by the Ottawa Police Services Board. The training curriculum is co-developed with the Ottawa Police Service's Professional Development Centre. The salary range is documented at approximately \$80,893 to \$95,168 as of 2025. Complaint process, oversight structure, and annual reporting to the Ottawa Police Services Board are all operational. This program is the closest existing Ottawa analogue for a Hospital Special Constable training pathway.

The Alberta AHS Benchmark

The AHS Community Peace Officer Induction Training Program is a six-week paid program co-administered with Alberta Justice and Solicitor General. The curriculum includes legislation, crisis intervention, control tactics, investigation, emergency response, and communication. Officers then complete field training before independent deployment. Recertification and use-of-force review are built into the program.

The Ontario Proposal

The training framework for the proposed Ontario Healthcare Special Constable designation is structured around four requirement categories, each mapped to Ontario Police College standards and adapted for the clinical environment:

- **Legal foundation.** OPC standards on Mental Health Act authority, patient rights, *Health Care Consent Act*, CSPA, documentation, and reporting obligations.
- **Clinical integration.** Hospital protocols, clinical handover procedures, Code White interaction, clinical communication, and electronic health record interface.
- **Use of force.** OPC standards, clinically adapted, with body-worn camera protocols, proportionality doctrine, and de-escalation as the primary response.
- **Cultural safety, trauma-informed care, and anti-racism.** Embedded throughout the program rather than isolated as a separate module, with specific attention to Indigenous-specific trauma and Black mental health.

FULL PATHWAY AND GRANDFATHER PATHWAY

A full entry-level pathway is proposed at six weeks minimum, consistent with both the OC Transpo and AHS precedents. A grandfather pathway — for officers with three or more years of hospital Protection Services experience, existing use-of-force certification, and a clean conduct record — is proposed at zero to four weeks supplementary training, subject to a competency assessment. The detailed curriculum is completed in partnership with the Ontario Police College as a pilot prerequisite.

ANNUAL RECERTIFICATION

Mandatory annual recertification across all four categories, quarterly use-of-force review, and mandatory incident debrief protocols. These are not optional. They are the condition of continued designation.

Four Steps. One City to Start.

The implementation pathway has four components. Each has a specific named owner. Each has a specific expected outcome. The detailed governance, oversight, and evaluation design is specified in Paper III.

Step One — OPSB Initiates the Designation Process

The Ottawa Police Services Board already has statutory authority under CSPA Part XIV to appoint Special Constables. The mechanism is operational today through OC Transpo. The first step is initiation of the designation process for a defined pilot cohort at QCH and TOH, pursuant to Solicitor General approval. **No new provincial legislation is required to begin.**

Step Two — Provincial Working Group Is Commissioned

The Ministry of the Solicitor General commissions a provincial working group, with membership drawn from frontline Protection Services Officers, hospital administration, clinical leadership, patient advocacy organisations, and Indigenous and racialised community representatives. The working group's mandate is the development of the provincial training, oversight, and accountability framework that would accompany a designated role beyond the pilot.

Step Three — EMCPA Modernisation Record Is Amended

Bill 25, the Emergency Management Modernization Act, 2025, is the most significant reform of Ontario's emergency management framework in two decades. The pre-triage hospital authority gap was not raised during the consultation. A formal amendment to the EMCPA record — identifying the gap as an unmitigated critical dependency in hospital emergency management planning — is the most direct available pathway to provincial acknowledgment.

Step Four — Defensive Equipment Standards Review

Parallel to the legislative and training work, a provincial review of defensive equipment and use-of-force tools available to Ontario hospital Protection Services personnel is initiated. The current gap between the documented risk exposure of this role and the equipment provided to manage it is a discrete, correctable problem. Minimum equipment standards for designated Healthcare Special Constables are established within the pilot, and the review considers whether a subset of those standards should apply to all hospital Protection Services personnel regardless of designation status.

Has This Been Introduced in Ontario Before?

No formal legislative proposal for a hospital-specific Special Constable designation has been tabled at Queen's Park. The 2019 HSJCC Police-Hospital Transition Framework is the closest Ontario has come to a structured provincial approach — and it deliberately addressed transition processes, not underlying authority. Various hospital associations and security professional organisations have raised the issue in stakeholder submissions without achieving legislative traction.

That absence is, in one sense, the opportunity. This series — *Hold the Line*, *Protecting the Protectors*, and *The Space Between* — is, to the author's knowledge, the first comprehensive, evidence-based, frontline-authored submission that frames the issue in terms of the existing Special Constable framework, uses the OC Transpo model as a local precedent, addresses the fiscal constraints explicitly, and specifies an implementation pathway with binding evaluation criteria.

// 08 – THE POLITICAL CASE

Bipartisan By Design.

The reason policy proposals of this kind have historically struggled to achieve legislative traction is that they get absorbed into political frames that do not fit them. A Healthcare Special Constable designation is simultaneously a public safety issue, a healthcare quality issue, a fiscal responsibility issue, and a worker protection issue. That means every party at the table has a principled reason to support it — and no party has a principled reason to oppose it.

// FISCAL
RESPONSIBILITY

No net new public spending.

Reallocation of existing capacity. No net new hospital staffing. Documented police resource recovery through the OPS CIT precedent. Earlier clinical engagement reduces expensive ED utilisation. The pilot's capped envelope limits fiscal commitment until outcome is demonstrated.

// HEALTHCARE QUALITY

Faster pathway to care.

Faster bridging from psychiatric crisis to clinical assessment. Improved patient experience, with a clinically-integrated professional rather than a uniformed police presence as the first on-site interaction. Reduced ED disruption. A workforce legislatively empowered to act as the clinical partner it already is in practice.

// PUBLIC SAFETY

Trained professionals in the gap.

Legislatively empowered professionals present in the interval between crisis and criminal incident. Reduced escalation pressure. A model that structurally prioritises care over custody because the clinical environment requires it.

// WORKER PROTECTION

Legislative recognition of the role.

Formal recognition of a profession absorbing the documented highest assault rate in the ED workforce. Legislated training standards and equipment requirements that match the actual risk. An alignment with existing nurses' union advocacy and workplace violence reform agendas.

// THE POLITICAL OBSERVATION

Every party in Ontario has committed, in recent policy statements, to reducing hallway healthcare, improving mental health outcomes, and using public resources responsibly. This proposal advances all three simultaneously. What remains is the political choice to act on a proposal that contradicts no party's platform and advances several.

To the People Who Do This Work.

I want to close this paper the way a frontline paper should close: by speaking directly to the workforce it is written about, and written from.

This profession has a cumulative weight problem. Anyone who has worked enough shifts in a busy Ontario hospital knows it. The frustration is real and it comes from legitimate ground — from people who are capable, committed, and proud of what they do, asked to absorb the weight of being treated, in writing and in organisational culture, as less than what the work actually requires. That weight produces cynicism, and bitterness, and the internal friction that makes hard shifts harder than they already are.

What I want every one of those people to understand is this: that frustration is not evidence that the role is not worth doing. It is evidence that the role has not been legislatively recognised. And legislative non-recognition is not a personal failure. It is a structural one, with a documented cause, a documented solution, and now a formal policy series making the case for that solution to the people with the authority to act on it.

We have built, in the major Ottawa acute care facilities, one of the most operationally demanding Protection Services environments in the country. We respond to situations that would challenge seasoned officers in any context. We do it with inadequate legal authority, incomplete equipment, and an organisational culture that has, in places, internalised its own diminishment. The fact that we do the work as well as we do is not evidence that the current framework is acceptable. It is evidence of what we are capable of when the framework is not acceptable.

The people at QCH — and at TOH, and at every Ontario hospital with an in-house Protection Services team — are not "just security." They are professionals who have chosen, deliberately, one of the harder environments in the protection services field, and who show up for it every shift, including the nights when the system fails the patient and you have to watch it happen and come back the next day anyway. That is what a professional does.

This paper exists because that commitment deserves a legislative framework that reflects it. The gap is documented. The solution is proven. The pathway is mapped. The next step is making the case public, and making it loud enough that it cannot be set aside.

// THE CLOSING CLAIM

This is not about power. It is about equipping the right people — the people who are already doing this work, every night, at the outer edge of what the current law permits — with the authority and the tools they need to do it safely, effectively, and without having to rely on circumstance.

The Specification Is in The Space Between.

Hold the Line named the urgency. Protecting the Protectors has made the frontline case. What remains is the full policy specification.

The Space Between — Paper III of this series — runs to approximately fifty-four pages. It contains: the three-stage failure model; the complete legislative authority map; the Charter analysis across sections 7, 9, and 15; the clinical primacy enforcement framework with eight specific operational rules including the fallback protocol for clinical unavailability; the training architecture mapped to Ontario Police College standards; the Ottawa two-site pilot design with primary and secondary endpoints; the pilot containment framework with automatic review triggers and mandatory off-ramp conditions; the full fiscal analysis anchored to police time reallocation as the primary offset; and the specific call to action directed to each of the four named recipients of this series.

Paper I was the case for attention. Paper II is the case for the mechanism. Paper III is the case for how to build it, govern it, evaluate it, and — if it does not meet outcome criteria — end it cleanly without further fiscal commitment.

Read the specification. Raise it in committee. Commission the working group. Begin the CSPA Part XIV designation process. The evidence base for action is complete. What remains is the decision to act on it.

— JORDAN ANDREWS · PROTECTION SERVICES OFFICER · QCH OTTAWA

